Everolimus + exemestane

Both tablets are taken every day continuously. There is no planned week off. A four-week blood-test review is a monitoring interval, not a stop-and-restart treatment cycle.

Oral targeted therapy + hormone therapy · selected medicine-specific effects. Full details and sources are in the complete reference.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.

1 Mouth soreness

everolimus

Often develops early; report ulcers before eating or drinking becomes difficult.

3 Blood glucose and lipids

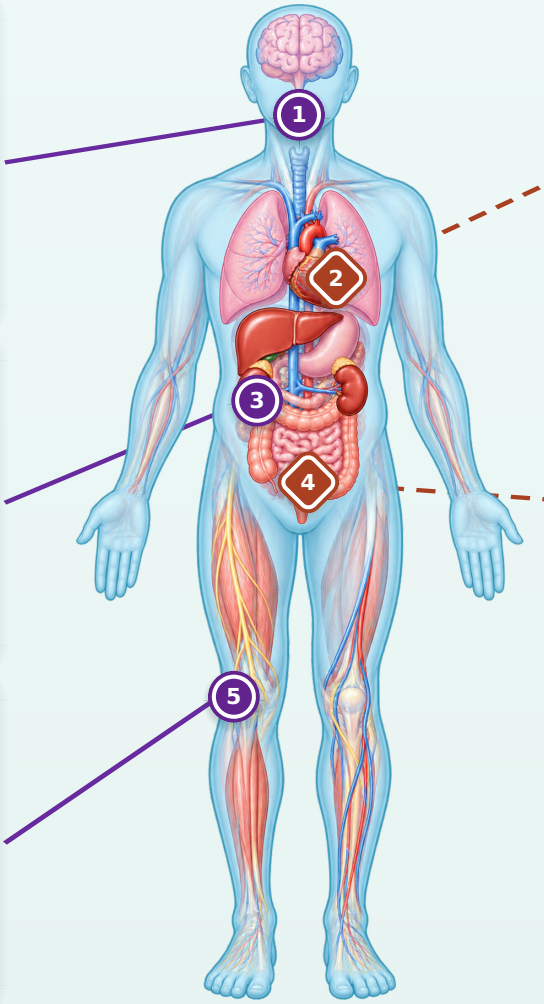
everolimus

Changes may be silent; keep blood-test appointments and report marked thirst or frequent urination.

5 Joints and bone health

exemestane

Joint symptoms can affect activity; bone loss needs risk-based monitoring.



2 New cough or breathlessness

everolimus

Prompt assessment is needed for pneumonitis, infection and other causes.

4 Diarrhoea or reduced intake

everolimus

Report persistent diarrhoea, inability to drink or dehydration.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Contact the oncology team promptly for new or worsening cough or breathlessness, fever or infection symptoms, mouth sores that prevent drinking/eating, severe diarrhoea or dehydration, or marked thirst and frequent urination. Severe breathing difficulty, collapse or swelling of the tongue/throat needs emergency help. Severe chest pain, major breathing difficulty, collapse or stroke symptoms: 112 / 999.

YOUR PRACTICAL TREATMENT PLAN

Know the next step.

Taking treatment

Both tablets are taken every day continuously. There is no planned week off. A four-week blood-test review is a monitoring interval, not a stop-and-restart treatment cycle.

Take everolimus at the same time daily, consistently with food or consistently without food. Take exemestane after a meal. Swallow tablets whole with water; do not crush or chew. If a dose is missed, take the next scheduled dose without adding an extra dose.

Checks and follow-up

Before starting: FBC, renal and liver profile, urea/BUN, urinary protein, blood glucose, cholesterol and triglycerides. Confirm the endocrine indication, review lung symptoms and infections, and check all medicines. Assess bone/fracture risk for the exemestane component. FBC, renal/liver profile, cholesterol and triglycerides every four weeks or as clinically indicated. Check fasting glucose as clinically indicated, with closer monitoring if diabetes or another medicine increases risk. Ask about mouth soreness, infection, cough/breathlessness and daily function at reviews.

The whole-treatment map shows the checks for this exact treatment setting.

Support between reviews

Nausea support

NCCP classifies everolimus as minimal-to-low emetic risk. There are no routine infusion premedications. Any as-needed antiemetic follows the individual prescription and local oral-SACT policy.

Prevent and treat mouth soreness

Discuss non-alcoholic mouth care and a prescribed alcohol-free corticosteroid mouthwash during the first eight weeks. Avoid irritating products containing alcohol, hydrogen peroxide, iodine or thyme derivatives. Check for oral thrush; do not automatically add an antifungal.

Steroids and infection prevention

Systemic steroids may be needed for pneumonitis. When steroids or other immunosuppressive medicines are used, the team considers PJP prevention; it is not an automatic prescription for every patient.

Glucose, lipids and bone health

Manage diabetes and lipid abnormalities alongside treatment. Bone-health advice and medicines depend on fracture risk, DXA findings, diet and vitamin-D status.

Medicine and supplement review

Check prescriptions, over-the-counter medicines and supplements with oncology pharmacy. Avoid grapefruit and St John’s wort; some antibiotics and antifungals can markedly change everolimus exposure.

Oncology daytime: Insert numbers here. **Out of hours:** Insert numbers here. Follow your own hospital’s urgent-care plan. Emergency: 112 / 999.



Your complete treatment reference

https://sactcheck.com/patient/00322/

Public information only. No personal information is encoded.